

Committee: United Nations Office on Drugs and Crime

Topic: The Question of Combating the Misuse of Medical Drugs in Post-Surgical Care and Chronic Pain Treatment

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Introduction

In recent years, the use of medical painkillers, especially opioid drugs, for post-operative recovery, chronic pain relief, and management has become widespread worldwide. These drugs and painkillers have a strong analgesic effect, providing physical and psychological relief to many patients suffering from various diseases. They are known to exert a strong analgesic effect by binding to proteins in the brain and spinal cord to reduce pain perception. Yet, they can also induce serious health problems associated with them, such as dependency, misuse, addiction, overdose, etc. Even if a drug is used for medical purposes, it is very easy for patients and users to become addicted to it, and it also carries the risk of repeated use, making it a major health issue and an international problem with social and economic ramifications that go beyond a simple medical procedure. The number of deaths from opioid-related drugs is increasing year by year, and the fact that many patients who need addiction treatment are not receiving appropriate treatment shows that this problem is not just a medical issue, but requires a structural response on an international level.

This problem is particularly serious in the United States, where it is called the ‘Opioid Crisis’. According to the US Centers for Disease Control and Prevention (CDC), more than 80,000 people died from opioid overdoses in 2021. However, this crisis is no longer limited only to the United States, but all around the global society. Overprescription and illegal distribution of medical drugs are becoming increasingly prevalent in countries such as Canada, Australia, the United Kingdom, and even in developing countries with looser regulations. To address these issues, the United Nations Office on Drugs and Crime (UNODC) and the World Health Organization (WHO) are promoting policies that seek a balance between safe and reasonable access to medical opioid drugs and the prevention of non-medical abuse after surgery and for treatment. These include strategies to increase

accessibility to anti-addiction drugs, regulatory guidelines that reflect country-specific conditions, development of medical staff education programs, and institutional support to improve medical accessibility.

Related to the previous paragraphs, and also for the chair report as a whole, the Declaration of Montreal, adopted at the International Pain Summit in 2010, emphasized that access to pain management is a fundamental human right. The declaration specifically states "the right of all people to have access to pain management without discrimination" and declares that failure to provide proper pain relief may constitute a violation of human rights. The declaration urges governments of all nations and medical institutions to establish systems and laws that ensure access to effective pain relief, including the appropriate use of opioids when necessary. This especially highlights the importance of not only dealing with abuse but also guaranteeing access to pain treatment for patients in need.

Meanwhile, societal perceptions of pain management are also part of the problem. Many patients believe they have a right not to have to endure pain, and some medical staff actively prescribe painkillers with the patient's satisfaction in mind. On the other hand, there are also cases where treatment is eliminated or chronic pain patients are denied access to necessary medication due to fears of addiction or the regulations in specific countries. These polarized perceptions make it even more difficult to formulate effective regulations, and finding the right balance between individual suffering and public health has emerged as an important international issue.

Thus, the problem of medical drug abuse is not simply a matter of the strength of regulations or prescription standards, but an international issue that is intricately intertwined with each country's medical system, social awareness, educational level, drug stigma, and legal system structure. Accordingly, WHO and UNODC are aiming for a comprehensive approach that sets global principles and standards while reflecting the particularities of each region, rather than a single solution. This agenda aims to explore how the international community can provide balanced solutions that can alleviate patients' pain during post-operative and chronic pain treatment processes while preventing the other scourge of drug addiction and abuse.

Definition of Key Terms

Prescription (Medical) Drugs

Strong central nervous system drugs are used for therapeutic purposes, such as pain relief, anesthesia, cough suppression, and diarrhea treatment, but they are subject to strict legal restrictions due to their potential for dependence and addiction. Internationally, opioid family drugs are classified as representative medical drugs, examples of which include Morphine, Fentanyl, Oxycodone, and Codeine. Because these drugs pose a strong risk of dependence and addiction, the 1961 United Nations 'Single Convention on Narcotic Drugs' allows their use internationally only for medical and scientific purposes, and all processes, including production, import/export, storage, and prescription, are strictly regulated. Meanwhile, the classification system for medical drugs differs from country to country, and drugs with the same ingredients may or may not be classified as prescription drugs depending on the country, and usability and prescription conditions may also differ.

Opioids

One of the most powerful and prevalent painkillers that act on the brain and nervous system to relieve pain, and are produced in natural, extracted from opium, or synthetic form. Examples of opioids include morphine, fentanyl, oxycodone, and codeine. Medically, they are used to treat severe pain, post-operative recovery, and cancer pain, but they carry a high risk of physical and psychological dependence and addiction, and can lead to serious side effects and death when abused. Therefore, most opioids are strictly controlled according to international treaties and national laws.

Opioid Stewardship

A policy and implementation strategy to systematically manage the appropriate and safe use of opioid analgesics at the institution or national health system level. The program aims to reduce overprescription, misuse, and dependency issues while ensuring adequate pain control for patients who need it. Core components include education of medical staff, creation of standardized prescribing guidelines, an Electronic Medical Record(EMR) based medication tracking system, patient monitoring, and guidance on medication use after patients are discharged from the hospital.

Fentanyl

Fentanyl is a powerful opioid analgesic, more than 100 times stronger than morphine and about 50 times stronger than nalbuphine. Because it has relatively weaker respiratory depression effects, it is used as an anesthetic adjunct or pain reliever, with its effects lasting 1 to 2 hours. It is mainly prescribed to patients who experience persistent pain, such as those with terminal cancer. Fentanyl is also widely used in the form of patches that can be applied to the skin, with the dosage carefully controlled at a micro level. However, fentanyl is highly addictive, and even a tiny amount, with just 2mg, can be fatal to a healthy person. In recent years, its misuse has surged as it has spread as a recreational drug in some countries. In 2021, fentanyl was reported to be the drug responsible for the highest number of overdose deaths in the United States, with over 70,000 people dying from fentanyl overdoses in that single year.

Naloxone

Naloxone is an opiate antagonist developed by Daiichi Sankyo Pharmaceuticals in 1960. It is specifically a drug used to reverse and build up barriers to an opiate overdose. It is particularly effective in treating life-threatening central nervous system and respiratory depression, but also has been used experimentally to treat Congenital Insensitivity to Pain with Anhidrosis (CIPA), which is a rare genetic disorder that causes the patient to lose the ability to sense, such as feeling pain, temperature, sweat, etc.

Complex Regional Pain Syndrome (CPRS)

Also called CPRS, Complex Regional Pain Syndrome is a chronic pain disorder that can occur after surgery or trauma, and is often difficult to control with over-the-counter painkillers. For this reason, some patients use prescription opioids, but these are limited to a last resort due to the risk of addiction and dependence. In most cases, anticonvulsants, antidepressants, physical therapy, and nerve blockade take priority, with opioids being used cautiously when other methods are ineffective.

Cannabis

Cannabis is a substance obtained by drying or processing the flowers and leaves of the cannabis plant, also known as Cannabis sativa, and typically contains the psychoactive component

tetrahydrocannabinol (THC). THC acts on specific receptors in the brain, causing effects such as hallucinations, mood elevation, and relaxation, which has led to widespread recreational use of cannabis. However, these effects can also lead to negative consequences such as dependence, impaired cognitive ability, and poor mental health, and cannabis is therefore strictly regulated in many countries. At the same time, research and use of cannabis are expanding due to its medical potential. Reports of its medical benefits, particularly those related to chronic pain, relief from vomiting and nausea in cancer patients, muscle relaxation in multiple sclerosis, and appetite stimulation, have led some countries to legalize its use for medical purposes. However, concerns about safety and abuse remain, and most countries permit medical cannabis only under strict conditions. Ultimately, cannabis is a substance that is viewed with conflicting views depending on the social, cultural, and legal context of each country, with both recreational and medical uses.

Single Convention on Narcotic Drugs (1961)

A UN treaty that created the first international framework to regulate narcotics, allowing medical use of opioids, but places them under strict global control via UNODC and the International Narcotics Control Board(INCB).

Background Information

Opioid Crisis

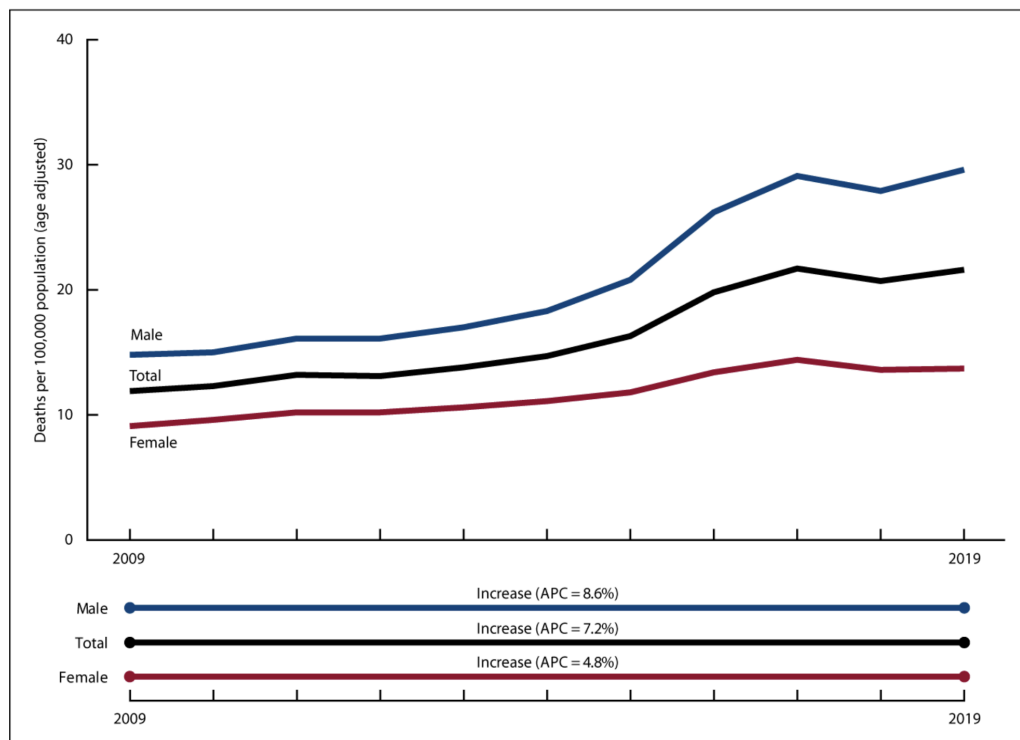
The opioid crisis refers to a social problem that has arisen since the 1990s, where the number of deaths due to abuse, misuse, overdoses, and poisoning of opioid-related drugs has increased exponentially. Opioids are powerful painkillers that are originally used during postoperative recovery and to treat chronic pain. They act on the central nervous system to relieve pain, but at the same time, they are highly addictive. In the US, Purdue Pharma launched OxyContin, a sustained-release formulation of oxycodone, in 1996 and conducted extensive marketing with the false claim that it had a low risk of addiction. During this process, lobbying interfered with the FDA (Food and Drug Administration) approval process, leading many doctors to readily prescribe opioids even to patients with relatively mild pain.

It is common to describe the opioid epidemic in three distinct waves. The first wave started in the 1990s with the prescription opioid epidemic, where the use of powerful prescription opioids resulted in uncontrolled addiction and inadequate pain management. The second wave started in 2010 and was marked by heroin use as a widely available substitute for prescription opioids. And finally, the last and most dangerous wave is the proliferation of synthetic opioids like fentanyl, which is far more powerful than prescription opioids. It is also known to be often sold unknowingly mixed with other illicit drugs. The crisis is still not yet resolved completely, as addiction is now more unrestrained than ever, and the emergence of new synthetic drugs has made the problem a whole lot worse. It is, however, a relief to see a decline in opioid-related deaths and the ongoing circumvention of the crisis. This issue prompted governments and communities to file lawsuits against pharmaceutical companies, which have resulted in multi-million dollar settlements aimed to be used in treatment and prevention.

Consequently, there have been many cases where patients who actually had a medical need became addicted after long-term use and turned to illegal drugs after their prescriptions were discontinued. According to the National Institute on Drug Abuse (NIDA), as of 2017, an average of 130 people died each day from narcotic painkiller overdoses, more than the number of deaths from traffic accidents and firearms.

As this situation intensified, many state governments in the United States filed a class action lawsuit against Purdue Pharma in 2007, and the company ended up filing for bankruptcy after paying

a settlement of about \$600 million. This situation was not simply an individual case of drug abuse, but was deemed a structural problem involving pharmaceutical companies, regulatory agencies, the medical system, and illegal distribution networks, and is today known as the ‘Opioid Crisis’.



Drug Overdose death rates, by sex: United States, 2009-2019

Take-Home Naloxone Program

The Take-Home Naloxone Program is a publicly funded emergency response program aimed at preventing deaths and brain damage from opioid overdoses. The program distributes the drug naloxone and provides education on how to recognize the signs of an overdose and respond appropriately. The program's focus is on providing kits and training to at-risk groups, particularly those taking opioids for post-operative pain medication or chronic pain treatment, as well as their families and friends, who may be at risk of overdose. Naloxone is an antidote to the respiratory depression caused by opioid drugs such as morphine, heroin, methadone, oxycodone, and fentanyl, restoring breathing and consciousness after an overdose. This drug binds more strongly to opioid receptors in the brain than opioid drugs, blocking their effects. It usually takes effect within five

minutes and lasts for 30 to 90 minutes. However, if a long-acting or high-dose drug is involved, the overdose can return after the effects of naloxone wear off, so 911 should be called and additional medication considered. For this purpose, the Take-Home Naloxone Kit contains a two-dose supply.

The program is designed to make naloxone accessible to everyone, providing simple yet effective education so that even non-medical professionals can also acknowledge and recognize an overdose and administer first aid. Education covers overdose risk reduction, overdose classification for opioids and other parasites, reporting to 911 in an emergency, rescue breathing, maintaining the recovery position, and the order of administering naloxone. This is not a substitute for professional medical services, but rather a procedure designed to save lives until arrival at a hospital. In particular, Saskatchewan provides kits and education free of charge to high-risk groups and those in the vicinity, and naloxone is available for purchase at pharmacies. Naloxone has been used in Canada for the past 40 years and is a highly safe drug listed on the WHO Essential Medicines List. While naloxone does not cause addiction and may induce withdrawal symptoms in opioid addicts, the low dose included in the Take-Home kits reduces this risk.

This program is now widely implemented across Canada and is being expanded as part of public health strategies in various countries, including the United States, Australia, and the United Kingdom. With the rise in deaths from high-risk opiate drugs, particularly fentanyl, increasing the availability of naloxone has become a key response worldwide. Several studies have shown that these programs actually save lives and reduce the burden on healthcare systems. Providing naloxone and educating patients, particularly those discharged from the hospital after surgery and those with chronic pain, is crucial, as these groups are at risk for drug overdose. This program goes beyond simple drug distribution, expanding emergency response capabilities throughout the community and serving as a vital public health pillar, providing education and connection for drug abuse prevention.

Medical Cannabis

Medical cannabis or medical marijuana refers to cannabis products and cannabinoid molecules prescribed by physicians to patients. While the use of cannabis as a medicine has a long history, clinical research to define the safety and effectiveness of using cannabis to treat illnesses is limited because it has not been as rigorously tested as other medicinal plants due to legal and government restrictions.

In theory, cannabis can reduce nausea and vomiting during chemotherapy and reduce chronic pain and muscle spasms. Regarding non-inhaled cannabis and cannabinoids, a 2021 review showed they offer little relief for chronic pain or sleep disorders and can cause several temporary side effects, including cognitive impairment, nausea, and drowsiness. Many cultures have used hemp for therapeutic purposes for thousands of years, and some U.S. medical organizations have called for hemp to be removed from the controlled substances list, emphasizing that rescheduling would ensure safe access through more extensive research and regulatory oversight. Some, such as the American Pediatric Association, oppose legalization. Medical cannabis can be administered in a variety of ways, including capsules, tablets, tinctures, skin patches, oral or dermal sprays, edible cannabis products, and vaporizing or smoking dried buds. Synthetic cannabinoids, such as synthetic delta-9-THC and nabilone, are available by prescription in some countries.

Countries that allow the medical use of whole plant cannabis are Argentina, Australia, Canada, Chile, Colombia, Germany, Greece, Israel, Italy, the Netherlands, Peru, Poland, Portugal, Spain, and Uruguay. In the United States, 38 states and the District of Columbia have legalized cannabis for medical purposes, beginning with the passage of California's Proposition 215 in 1996. While cannabis remains prohibited for all uses at the federal level, the Rohrabacher-Pall Amendment was enacted in December 2014, limiting the ability of states that have legalized medical cannabis to enforce federal law. This amendment reflects increasing recognition in both parties of the potential therapeutic uses of cannabis and the importance of state-level policymaking in this area.

Declaration of Montreal

The Declaration of Montreal is an international declaration established by the International Association for the Study of Pain (IASP) in 2010, which stipulates that all human beings have equal access to pain treatment as a fundamental human right. This declaration starts from the reality that many patients around the world are unable to receive effective pain management treatment, and emphasizes that, in particular, in low-income countries and regions with strict regulations, pain relief treatment, including narcotic analgesics, hardly reaches patients. And it contains the awareness that this should not be regarded as merely a lack of medical services, but as a human rights violation.

The declaration emphasizes three major human rights violations: First, ‘it is a human rights violation if patients cannot receive appropriate treatment even while in severe pain’, second, ‘it is also considered a human rights violation if medical personnel responsible for pain treatment are not provided with sufficient education and resources’, and last but not least, ‘it is also defined as a serious

human rights violation if the government prevents patients from accessing necessary medicines through unnecessarily strict regulations”. Thereby, it defines and emphasizes the content of treatment ethics for all human beings. This awareness makes it clear that this is not merely a medical problem, but a fundamental right that society as a whole must guarantee, and shows that people can receive treatment fairly and of their own free will.

As a result, the declaration calls on governments and health authorities in each country to remove institutional barriers to pain management and to guarantee reasonable access to therapeutic drugs, including narcotic analgesics. In particular, although opioid analgesics carry a risk of misuse, if they are not provided to patients who need them, it results in unnecessary suffering being left untreated. Therefore, the declaration presents a balanced perspective that both preventing misuse and ensuring access must be achieved at the same time, and this functions as an important reference point in today’s discussions on medical narcotics management.

Possible Solutions

Stepwise Prescribing Principles for Chronic Pain Patients

Strategies to reduce opioid use are not sufficient by simply limiting doctors' prescriptions. In order to adequately control the patient's pain while preventing unnecessary long-term use, systematic and evidence-based clinical guidelines are necessary. For example, the clinical guidelines announced by the U.S. Centers for Disease Control and Prevention(CDC) in 2022 distinguish between acute, subacute, and chronic pain and present recommendations according to each situation. In particular, for patients with acute pain, it is recommended to prescribe for a short period of usually within 3 days, and no more than 7 days, and instead of starting with a high dose from the beginning, start with the minimum effective dose and adjust step by step as needed. These guidelines play an important role as a safety device to effectively control the patient's pain while preventing unnecessary long-term use.

In fact, several studies have shown that when multimodal analgesia is applied to patients immediately after surgery and nerve blocks or local anesthetics are used in combination, the analgesic effect for patients is sufficiently maintained while opioid use is dramatically reduced. The Enhanced Recovery After Surgery(ERAS) protocol, which is widely used in the UK and the US, specifies the recommended dosage for each type of surgery, and reports that compliance with this significantly reduces the incidence of leftover drugs. For instance, in the state of Michigan in the United States, the average opioid prescription required for patients after surgery was analyzed, and standard recommended amounts for each surgery were presented, after which the actual rate of leftover drugs was reduced by nearly half. This is evidence that systematic guidelines are not just theoretical recommendations, but actually contribute in practice to maintaining patient treatment effectiveness while alleviating social problems.

Expanding Treatment Accessibility

Just telling a patient with Opioid Use Disorder(OD) to stop using drugs for safety reasons is inadequate, especially when considering the connection to the severe pain symptoms they may be experiencing. Since the withdrawal symptoms inflict such severe pain physiologically and psychologically, the use of Medication for Opioid Use Disorder(MOUD) is necessary for effective treatment because of the high relapse rates. Some internationally recognized medications for MOUD

that reduce withdrawal symptoms and support long-term recovery and relapse prevention are buprenorphine, methadone, and naltrexone.

In 2023, the 'X-waiver' system was removed, and primary care doctors started to have easier access to prescribe buprenorphine, significantly increasing its availability, especially in the US rural areas and under-resourced areas. Despite the positive impact, the provision of MOUD in the absence of mental health counseling, social support, and rehabilitation services is not useful. One example is in Ontario, Canada, where a model was created to give patients methadone and concurrently provide them with vocational and psychological counseling, increasing not only retention rates but also treatment effectiveness.

Harm Reduction Strategies

The most direct consequence of the opioid crisis is 'death from overdose'. A representative harm reduction strategy to prevent this is the distribution of naloxone. Naloxone is an antidote that binds to opioid receptors and rapidly blocks the effects of the drug, and it is available in forms such as intravenous injection or nasal spray. In parts of Canada and some U.S. states, 'Take-home Naloxone Kits' are provided not only to patients but also to family members and friends, enabling immediate response in emergencies. In fact, overdose mortality rates have significantly decreased in these regions. WHO and UNODC also strongly recommend widespread community-level distribution of naloxone, and several low-income countries have begun dissemination with support from international organizations.

In addition, Needle Exchange Programs and Overdose Prevention Centers(OPCs) play a crucial role as hubs for preventing infectious diseases, reducing mortality, and linking patients to treatment. For example, the Insite center in Vancouver, Canada, is the world's first officially sanctioned safe injection facility. Under medical supervision, patients can use drugs safely while also receiving counseling and referrals to treatment. As a result of its operation, opioid-related mortality in the area has dropped significantly, and rates of HIV and hepatitis infections have also declined. These harm reduction strategies go beyond mere emergency response. Because they function as a 'gateway' for reconnecting patients to the healthcare system, they ultimately help promote rehabilitation and reintegration into society. Therefore, alongside expanding naloxone distribution, the international community needs to strengthen policy and financial support so that countries can adopt prevention center models suited to their local communities.

Safe Disposal of Medication After Use

Opioid medications left over after surgery are often left unattended at home, where they may be discovered by adolescents or family members, leading to misuse. To reduce such risks, the DEA in the United States regularly organizes ‘Drug Take-Back Days’ and operates ‘drop-boxes’ to encourage the safe collection of narcotics. Furthermore, some hospitals provide patients with home medication disposal guidelines or deactivation kits upon discharge in an effort to prevent drug misuse and diversion.

Prevention Through Pre-Education to Prevent Drug Misuse

As a method to prevent drug misuse in advance, there is likely no prevention measure as efficient and widely adopted as ‘education’. Through education, doctors and drug specialists need to inform patients not only about the dosage and duration of use but also about signs of overdose, withdrawal symptoms, and how to use naloxone. Furthermore, if the risks of opioids and alternative treatment options are included in preoperative explanations, patients can participate more actively in their treatment process, and the risk of drug misuse is also reduced.

Major parties involved

United States of America

From 1999 to 2021, approximately 645,000 people died from opioids in the United States, with overdose deaths in 2021 ten times higher than in 1999. The crisis began in the 1990s with the excessive prescribing of opioids for post-surgical pain and chronic pain management. At the time, pain was considered the 'Fifth Vital Sign', and doctors actively prescribed painkillers, while pharmaceutical companies like Purdue Pharma expanded marketing of a formula for OxyContin that reduced the risk of addiction. Between 1991 and 2011, prescriptions for painkillers more than tripled, with over 280 million opioid prescriptions being written in 2016. In the process, opioids were used not only for pain relief but also for pleasure-seeking and long-term use, giving rise to problems of addiction and dependency, and the strength of prescription drugs gradually increased.

In this way, prescriptions that began as legitimate medical procedures spread to non-medical use, leading to a sharp increase in drug abuse and deaths. The nature of the US healthcare system, in particular, which relies on cheap prescription drugs, increases the risk of opioid abuse among low-income earners, and the prescription rate in the US is 40% higher than in Germany or Canada. Meanwhile, while opioid prescriptions have increased, bio-opioid painkillers such as aspirin and ibuprofen have been used less and have not reduced the pain patients experience, raising medical questions about whether opioids are effective for chronic pain other than cancer pain. Ultimately, the problem of drug abuse is the result of a complex web of overprescription, mismarketing, and systematic undersubstitution, and safer, more responsible use of painkillers in post-operative management and chronic pain treatment is urgently needed.

The United States has recognized the opioid crisis as a national health crisis and has implemented comprehensive policies to address it. In 2017, President Trump officially declared the crisis a Public Health Emergency, launching a full-scale national response that has resulted in a multifaceted legal, institutional, and medical approach to reducing the abuse of narcotic painkillers. One key measure is the Prescription Drug Monitoring Programs (PDMPs), which are designed to allow healthcare professionals to review a patient's opioid prescription history in real time and prevent duplicate or overprescribing. PDMPs are legally mandated in most states and play a critical role in the prevention and early detection of opioid abuse.

Furthermore, the United States Centers for Disease Control and Prevention (CDC) published its 2016 opioid prescribing guidelines, recommending more careful and restrictive prescribing of opioids for chronic pain. The core content of these guidelines is that, except for patients with cancer or terminal illnesses, opioids should be prescribed as a last resort, after first considering available non-drug treatments and bio-opioid painkillers, and that when prescribing, the lowest possible dosage and shortest possible duration should be the principle. These guidelines are positioned as a benchmark for healthcare systems to minimize the risk of misuse and prevent potential patient dependency and addiction.

Canada

In 1993, a British Columbia Chief Medical Examiner's investigation determined drug-related deaths were disproportionately high, with 330 deaths reported at the time. Opioid-related deaths have since risen sharply, with 2,861 deaths across Canada in 2016, 1,473 in British Columbia in 2017, and 3,996 deaths across Canada. From 2016 to 2022, the death rate per 100,000 population more than doubled to 20.3 deaths per year, with 1,904 deaths already recorded in the first quarter of 2023. In 2015, Canada ranked second in prescription opioid use per person after the United States, and Alberta has seen a 1,000% increase in emergency room visits due to fentanyl and analogues over the past five years. As of 2017, 92% of Canada's 3,987 opioid-related deaths were accidental, with nearly half of overdoses intentional among adolescents aged 15-24. Deaths involving fentanyl or similar drugs increased 17% from the previous year.

Between April and December 2020, opioid-related deaths increased by 89% compared to the previous year, and Saskatoon recorded its highest monthly overdose rate due to a combination of increased toxic drugs and COVID-19 quarantine. Medical services were called out to approximately 28,800 opioid-related emergencies in that year alone, and naloxone was administered in over 250 cases in May. In 2003, Insite, the first safe injection room in North America, opened in Vancouver's Downtown Eastside, and 16 safe injection rooms were authorized nationwide by 2017. Such facilities provide harm reduction approaches such as on-site medical staff, provision of injection tools, counseling, and treatment coordination. In 2012, the prescription opioid OxyContin was removed from the list, which led to an increase in the distribution of the more powerful illegal drugs fentanyl and caffeine. As of 2018, approximately 1 million people were at risk of exposure to such toxic opioids. Moreover, since 2012, the Take-Home Naloxone Program (THN) has been operating to distribute naloxone to high-risk groups.

Canada recognizes the misuse and addiction of medical narcotic painkillers as a serious public health crisis and has established a strong regulatory and integrated management system to address this issue. The key is to ensure the appropriateness and safety of narcotic use by strengthening direct government control and monitoring, rather than relying solely on the judgment of prescribers. In particular, revised narcotics control regulations will come into effect on June 5, 2024, requiring all facilities that handle narcotics, including medical institutions, pharmacies, and research institutes, to obtain prior approval from the federal government and to systematically manage the entire process from distribution to storage, use, and disposal. Related records must be reported in real time to an integrated system.

China

The early drug-related laws in China defined the scope of drugs very narrowly, regulating only opium, heroin, morphine, and cannabis. However, as new substances and forms of drugs gradually appeared, the scope of controlled substances expanded. The current Narcotics Control Law covers opium, heroin, methamphetamine, morphine, cannabis, and all addictive narcotics and psychotropic substances designated by the state. According to Article 2 of the Narcotics Control Law, Article 3 of the Administrative Regulations, and the ‘Notice on the Main Responsibilities of the Member Units of the National Narcotics Control Commission by the General Office of the State Council, the designation of such narcotic drugs and psychotropic substances is handled by the State Drug Administration. The term ‘State Drug Administration’ here refers to the State Food and Drug Administration (SFDA), which is under the Ministry of Health. Based on Article 3 of the Administrative Regulations, the SFDA, in consultation with the public security and health authorities, designates and announces narcotics and psychotropic drugs.

The most recent ‘Catalogue of Controlled Narcotic Drugs and Psychotropic Substances’ was promulgated on October 11, 2007, and has been in effect since January 1, 2008. It designates and regulates 123 narcotics and 132 psychotropic substances. Among these, psychotropic substances are further divided into Category I and Category II according to their dependence potential and risk. Category I psychotropic substances are considered the most dangerous and are strictly controlled by the state from the production process, under the authorization and supervision of the SFDA. They cannot be sold at retail (Administrative Regulations, Article 30), and prescriptions are limited to one dose for injections, within seven days for sedatives, and within three days for other cases. Category II psychotropic substances can be sold at retail, but prescriptions are limited to within seven days. However, in cases with a physician’s opinion, prescriptions exceeding seven days are permitted. In

addition, there are differences between Category I and II psychotropic substances regarding use, management, authorization, prescription duration, and record-keeping. The current catalogue regulates 53 Category I psychotropic substances, including cathinone and diethyltryptamine, and 79 Category II psychotropic substances, including amobarbital and butalbital.

United Kingdom

Recently, the United Kingdom has seen an increase in cases of medical cannabis use violations. While the UK legalized cannabis for medical use in 2018, some private citizens believe it is legal and are even growing it at home to use as medicine. As the number of people arrested for illegal cannabis use increases, the number of lawyers specializing in cannabis cases is also on the rise. The UK magazine Time reported that there has been a sharp increase in illegal use due to ignorance since medical cannabis became legal in 2018. According to the International Journal of Pharmaceutical Policy, the UK government's lack of implementation of medical cannabis legalization is due to insufficient investment, bureaucratic supply issues, and problems with training doctors and police. The UK government has revised arrest guidelines to address the increasing number of cases, requiring patients to carry identification and a prescription when in possession of cannabis. In the UK, medical cannabis is permitted only for patients with serious conditions such as severe encephalopathy and multiple sclerosis, and approximately 89,000 prescriptions have been issued since 2018. The problem is that as prescriptions increase, so does the number of people using them, and the number of police crackdowns is also on the rise. According to Time, while exact figures have not been released, it has been found that law firms are seeing an increase in the number of cases involving marijuana. Robert Geppay, a lawyer specializing in marijuana, said, "We're receiving more and more reports every week of patients having their drugs seized or being arrested." While the government has permitted marijuana use by prescription only for patients, the public has begun to believe that marijuana can be used for medical purposes, leading to an increase in illegal cases, such as people growing and using their own marijuana.

The UK strictly regulates medical drugs under the Misuse of Drugs Act 1971 and the Misuse of Drugs Regulations 2001. Powerful opioids such as morphine and oxycodone are classified as Schedule 2, and all processes from prescription to storage and disposal are strictly controlled, with medical institutions and pharmacies required to keep all records with government permission. Additionally, in accordance with NHS and NICE guidelines, it is recommended that short-term prescriptions and fast-acting formulations be prioritized, and medical staff must assess not only patients' pain but also their risk factors for addiction. At medical institutions, drug managers are responsible for everything from drug purchases to disposal, and usage history is tracked in electronic

record systems. Strong sanctions will follow if illegal drug distribution or overprescription is discovered, based on the principle of ‘ensuring access to patients who need it, while thoroughly preventing abuse.’

Republic of Korea

In South Korea, fentanyl is prescribed to most patients undergoing major surgery, such as those with terminal cancer, severe CRPS, or fractures. If a patient experiencing severe pain due to a related illness receives outpatient treatment without being hospitalized, they are almost always prescribed fentanyl. It is prescribed in the form of a patch worn on the body, a spray sprinkled on the inside of the nose, or a sublingual tablet that dissolves in the mouth or under the tongue, much like a candy. However, with patches, there is a risk of over-absorption due to Korean lifestyles, such as heating the floor with a boiler or lying down on various heating mats in bed to keep the temperature up, which poses a problem. Sicker patients are more susceptible to colds, and parents often turn up the temperature of heating products to warm their patients, leading to numerous cases of fentanyl poisoning from patches.

Medical fentanyl is also a blinding poison, so it must be kept out of reach of children and animals and requires additional security measures, such as warning doors or locks, and informing residents. However, many stakeholders are opposed to extremely strict regulations, such as a complete ban on fentanyl or stricter limits on annual production. Clearly, there are people who are prescribed these painkillers and who struggle to survive without them, and refusing to prescribe them because of regulations is inhumane and wrong. The situation of those who need medication but are not being given it could spark even greater anger, so they are reluctant to speak out. Furthermore, in South Korea, fentanyl prescriptions for many pain conditions are included as a non-prescription item, placing a significant financial burden on patients, making further restrictions difficult. However, further restrictions have resulted in greater harm to patients. In fact, the anesthesiology and pain medicine departments of large hospitals that treat severe pain conditions are not even able to prescribe fentanyl properly. Furthermore, since 2020, large hospitals have even begun to stop prescribing fentanyl-based medications at the patient's expense for severe pain conditions such as CRPS, post-herpetic neuralgia, and severe fibromyalgia, and instead provide appropriate pain treatment at institutions that primarily treat severe pain patients.

Germany

Germany emphasizes multimodal pain management for post-operative and chronic pain treatment, incorporating the principles of ‘short-term, minimally effective doses when needed’ and non-pharmacological treatments. Long-term prescriptions are strictly restricted and evaluated, and the guidelines for Long-Term Opioid Use in Chronic Noncancer Pain (LONTS) recommend regular reassessment of benefits and risks and the establishment of discontinuation criteria. In recent years, there has been no clear increase in opioid prescriptions in Germany since 2016, and the rate of long-term prescriptions has been stable. This reflects a culture of careful prescribing and guideline-driven clinical practices to prevent overuse and addiction. At the hospital level, quality management regulations are now mandated to include a system for ‘appropriate postoperative pain treatment’, systematizing the use of painkillers during hospitalization and discharge, as well as education, risk assessment, and record management. Under this framework, Germany makes opioids accessible to patients who need them, while maintaining a balanced approach (compliance with clinical guidelines, minimizing dosage and duration, and prioritizing alternative therapies) to reduce the risk of misuse and addiction.

In terms of legal and institutional aspects, Germany strictly controls medical narcotics through its Narcotic Drugs Act (BtMG) and Narcotic Medicines Prescription Regulations (BtMVV). Doctors can only prescribe narcotic painkillers using a special three-part ‘BtM prescription’, and the entire process, including issuance, preparation, storage, and return, is tracked and recorded. The 2023 revision modernized some technical requirements, established requirements for alternative treatments, clarified the scope of telemedicine use, and removed the previous uniform maximum dose restrictions, thereby introducing flexibility based on patient safety. The prescriptions themselves are issued and distributed by the Federal Agency for Drugs and Medical Devices (BfArM), which manages a large number of BtM prescriptions each year. Furthermore, hospitals and clinics must now operate pain management, especially post-operative pain management, in accordance with revised G-BA (Federal Joint Commission) quality management regulations. Meanwhile, medical cannabis has been given a separate system under the Medical Cannabis Act enacted in 2024, which ensures accessibility to patients while also strictly controlling prescription, preparation, and distribution. Overall, the German government's position can be summarized as a triple safeguard of norms, monitoring, and quality control that simultaneously guarantees accessibility to medically justified pain treatment and systematically prevents misuse.

Timeline Of Events

Date	Description of event
<i>Adoption of the Single Convention on Narcotic Drugs</i> March 30, 1961	The UN established a global treaty framework that permitted medical use of narcotics but subjected them to strict international controls.
<i>FDA approval of OxyContin</i> December, 1995	The U.S. Food and Drug Administration approved OxyContin, whose aggressive marketing and overprescription became a major driver of the opioid epidemic.
<i>Establishment of the UNODC</i> 1997	The United Nations Office on Drugs and Crime (UNODC) was created through the merger of the United Nations Drug Control Programme and the Centre for International Crime Prevention.
<i>World Drug Report First Published</i> 1997	The World Drug Report is published annually by the United Nations Office on Drugs and Crime. The first report was published in 1997, the same year the agency was established.
<i>UN General Assembly Special Session (UNGASS) on Drugs</i> June 8-10, 1998	Member States adopted a Political Declaration and Action Plan, committing to eliminate or significantly reduce the illicit production, trafficking, and misuse of drugs within 10 years.
<i>Convention Against Transnational Organized Crime (Palermo Convention)</i> November 15, 2000	Entered into force with UNODC as the guardian body, addressing organized crime, including drug trafficking networks.
<i>Launch of the Paris Pact Initiative</i> May 22, 2003	UNODC and partners introduced the Paris Pact to combat opiate trafficking from Afghanistan, one of the largest global drug supply routes.
<i>Launch of North America's first Safe Injection Facility (Insite, Vancouver)</i> September, 2003	Canada established the Insite facility, pioneering a public health approach to overdose prevention and victim protection.
<i>UNODC launches 'International Standards for the Treatment of Drug Use Disorders'</i> 2009	Developed global guidance to support evidence-based treatment of drug dependence, emphasizing health-centered approaches.

<i>UNODC/WHO Programme on Drug Dependence Treatment and Care</i> March 11, 2009	A joint initiative began to integrate public health into drug control strategies and expand access to evidence-based treatments.
<i>UN Political Declarations on HIV/AIDS</i> June, 2011	The UN highlighted the imbalance between ensuring medical access to opioid painkillers and addressing their misuse, drawing international attention to dual challenges.
<i>UNGASS 2016 on the World Drug Problem</i> April 19-21, 2016	Member States recognized drug dependence as a public health disorder, stressing balanced approaches between supply control and demand reduction.
<i>CND Adopts Resolution on Access to Controlled Medicines</i> March, 2017	UNODC and the Commission on Narcotic Drugs (CND) emphasized the importance of ensuring medical availability of opioids for pain treatment while preventing diversion and misuse.
<i>U.S. Declares Opioid Crisis a National Public Health Emergency</i> October 26, 2017	The federal government formally recognized the opioid epidemic as a national emergency, triggering a coordinated response across states and agencies.
<i>UNODC launches 'Listen First' Campaign</i> November, 2018	Evidence-based prevention initiative focused on supporting youth, parents, and educators to reduce substance misuse.
<i>UNODC COVID-19 and Drugs Report</i> June, 2020	Highlighted how the pandemic exacerbated drug trafficking vulnerabilities and disrupted access to essential controlled medicines.
<i>World Drug Report 2021</i> June 24, 2021	Warned of a surge in opioid use and overdose deaths globally, stressing the urgency of balanced regulatory frameworks.
<i>Launch of UNODC Synthetic Drugs Strategy</i> November, 2021	Established a global framework to address the rise of synthetic opioids like fentanyl and methamphetamine.
<i>UNODC Technical Guidance on Opioid Crisis Responses</i> 2022	Issued recommendations for Member States on integrated strategies, including naloxone distribution, treatment accessibility, and surveillance systems.

UN Involvement, Resolutions, Treaties and Events

- The Single Convention on Narcotic Drugs (1961) is an international treaty that includes essential opioids regulated by the World Health Organization (WHO) and the International Narcotics Control Board (INCB). It guarantees access to opioids for pain relief while also including provisions to control misuse and distribution.
- The Convention on Psychotropic Substances (1971) is a treaty regulating psychotropic substances such as LSD and MDMA, encompassing the international regulatory framework for drugs other than opioids.
- UN Convention Against Illicit Traffic in Narcotic Drugs and Psychotropic Substances (1988) addresses drug-related crimes such as tax evasion, money laundering, and international smuggling, providing a legal basis for preventing the illicit trafficking and misuse of opioids.
- The 30th Special Session of the UN General Assembly on the World Drug Problem was held on April 20–21, 2016, during which member states reached an international consensus on health-centered policies and expanded access to pain management.
- The UN General Assembly Special Session (UNGASS) – ‘The World Drug Problem’ (2016) adopted a policy declaration and action plan emphasizing a health-centered approach. Recommendations included expanding access to medication-assisted treatment (MAT/MOUD), needle exchange programs, rehabilitation and social reintegration support, and access to essential medicines for pain management.
- The Commission on Narcotic Drugs (CND) adopted several resolutions to shift towards public health-based policies. In 2018, the UN Secretary-General emphasized the need for a ‘public health approach’, and subsequently ECOSOC and the General Assembly recommended health-centered approaches as international policy.
- UNODC Opioid Strategy (June 2018) is a multi-agency collaboration strategy developed by the United Nations Office on Drugs and Crime (UNODC) in cooperation with WHO, INCB, INTERPOL, and others. It is structured around five pillars: real-time alerts, rational

prescribing, prevention and treatment programs, international enforcement, and capacity building.

- On May 26, 2023, UNAIDS hosted the UN-led meeting ‘Responding to the Global Crisis of Overdose’, which brought together experts to share and discuss the current state of responses, policies, and innovative approaches addressing the global overdose crisis.
- In 2024, High Commissioner Volker Türk declared that the ‘war on drugs’ had failed and urged the international community to shift toward human rights and health-centered approaches, including harm reduction policies, instead of punitive measures.
- A CND side event (March 13, 2025), hosted by Canada in cooperation with UNODC, presented real-world responses to the opioid overdose crisis. UNODC’s five-pillar strategy served as the central focus of the discussions.

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